

Patient ID: TMillerPeriod: July 3 to October 1, 2024Report Date: October 2, 2024Coverage: 1608/2184h (74%)

Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Clinical Alerting Thresholds

Very Low HR < 40 bpm

Very High HR > 110 bpm

Low HR < 45 bpm

Elevated Breathing > 24 brpm

Elevated HR > 95 bpm

High Breathing > 30 brpm

High HR > 100 bpm

Very High Breathing > 40 brpm

Last 24 Hours

Elevated Breathing 11 AM to 12 PM		High Breathing 12 PM to 1 PM	
Vital Stat	Avg	Min	Max
Heart Rate	60 bpm	48 bpm	75 bpm
Breathing	18 brpm	11 brpm	53 brpm

Last 24 hours: two elevated breathing episodes in the morning; one high breathing episode in the afternoon.

Episodic Events (last 24h)

Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
11 AM to 12 PM	3h	3	Elevated Breathing	27	13	53	Check SpO2 and lung sounds; assess for fluid overload or early infection
12 PM to 1 PM	1h	1	High Breathing (brief)	32	22	47	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF

Patient clinical status over 91 days from Jul 03 to Oct 01

HRBreathing

Elevated Breathing Jul 26 to Jul 29 (4d)	Elevated Breathing Aug 08 to Aug 17 (10d)	Elevated Breathing Aug 20 to Aug 24 (5d)	Elevated Breathing Sep 11 to Sep 14 (4d)	Elevated Breathing Sep 30 to Oct 01 (2d)	High Breathing Jul 26 to Jul 27 (2d)	High Breathing Aug 15 to Aug 16 (2d)	High Breathing Aug 22 to Aug 24 (3d)	Low HR Jul 31 to Aug 03 (4d)	Low HR Aug 09 to Aug 15 (7d)	Low HR Aug 24 to Aug 30 (7d)
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70 episodic events Detected, spanning 4 days total. Conditions: Elevated Breathing, High Breathing, Low Heart Rate. Priority grade: Medium

Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
70	108h	Low Heart Rate	<1	74%

Major Findings: Sustained low HR (avg 43, min 40)

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
Jul 26	1 PM to 2 PM	3h	3	High Breathing	35	11	56	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
Aug 15	2 PM to 3 PM	2h	2	High Breathing	32	16	45	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
Aug 22	1 PM to 2 PM	2h	2	High Breathing	35	12	55	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
Jul 31	9 PM to 11 PM	8h	3	Low Heart Rate	42	40	47	Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
Aug 09	11 AM to 12 PM	24h	10	Low Heart Rate	43	40	48	Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
Aug 24	12 PM to 2 PM	28h	12	Low Heart Rate	43	40	48	Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic

+ 5 additional condition(s) across Elevated Breathing; details in monitoring data.

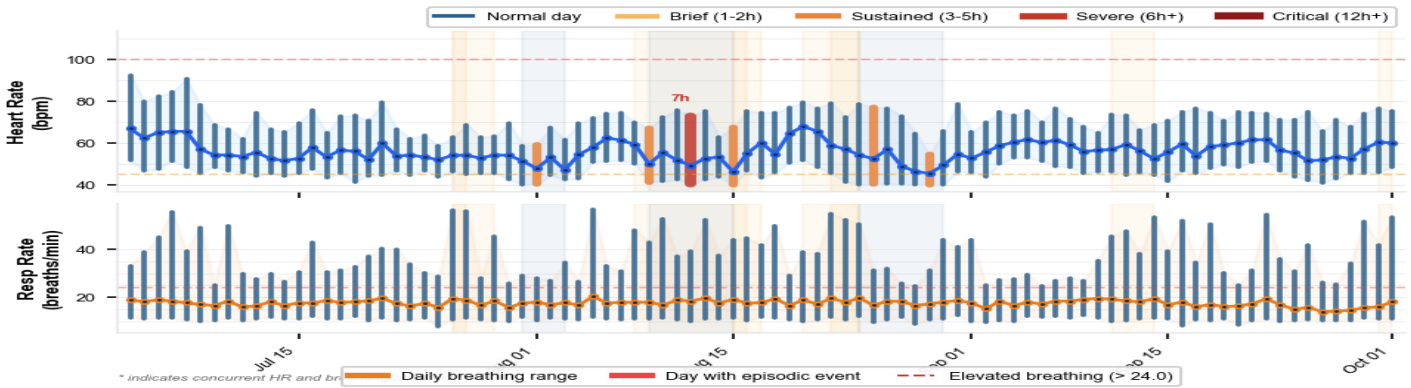
The P5 to P95 heart rate spread was 23 bpm (46 to 69), indicating significant cardiac variability.

Suggested Clinical Review Actions

- Low Heart Rate (avg 43 bpm, min 40 bpm): Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
- Elevated Breathing (avg 26 brpm, peak 53 brpm): Check SpO2 and lung sounds; assess for fluid overload or early infection
- High Breathing (avg 35 brpm, peak 56 brpm): Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
- Overall trajectory shows 11 unstable phases with 23 bpm variability. Consider increasing monitoring frequency and lowering escalation threshold.

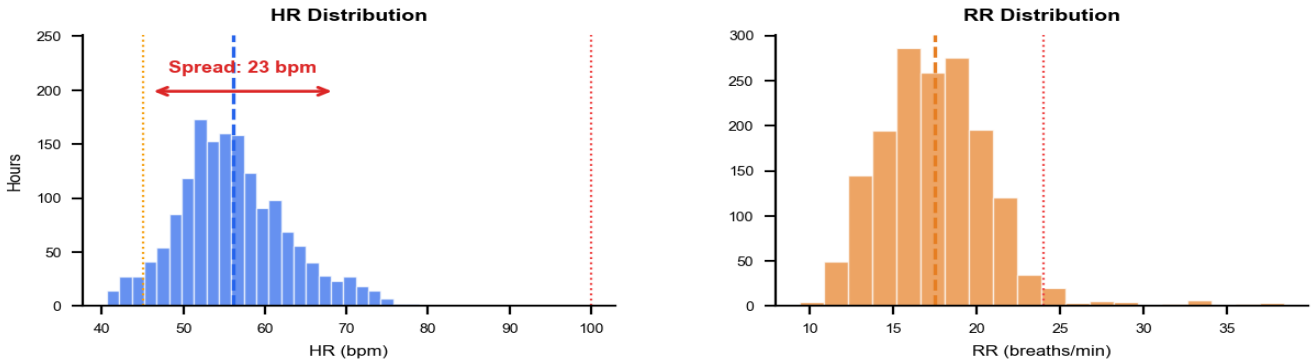
Clinical Pattern Observations:

- 1. **Sustained finding:** 7h continuous Low Heart Rate on Aug 12.
- 2. **High variability:** HR spread 23 bpm (45 to 68).
- 3. **Clustered pattern:** Episodic events on 6 of 91 days (6%).

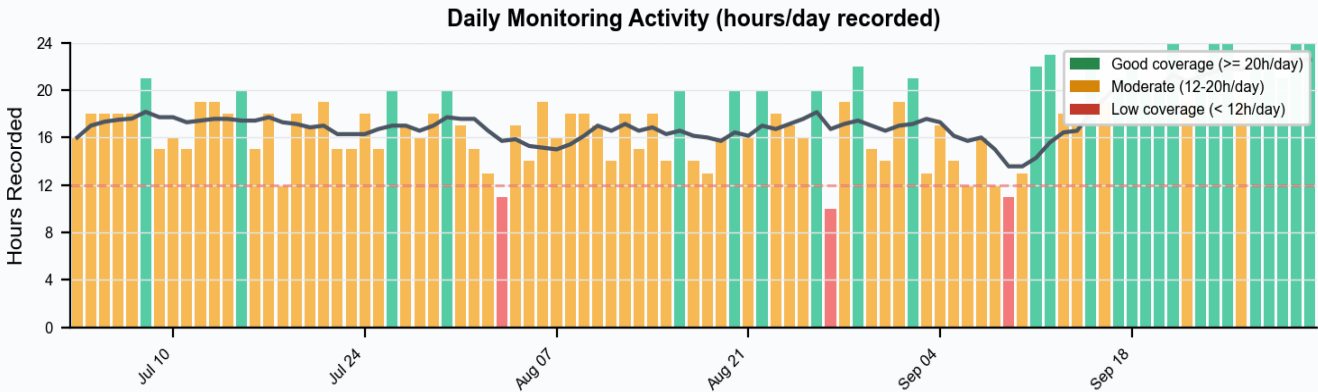


Red bars indicate days with detected episodic events.

Vital Sign Distribution



Monitoring Activity



Bars indicate monitoring hours/day. Red threshold line indicates clinical monitoring baseline.

■ HR episode ■ RR episode ■ Recorded, no episode ■ No data

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.